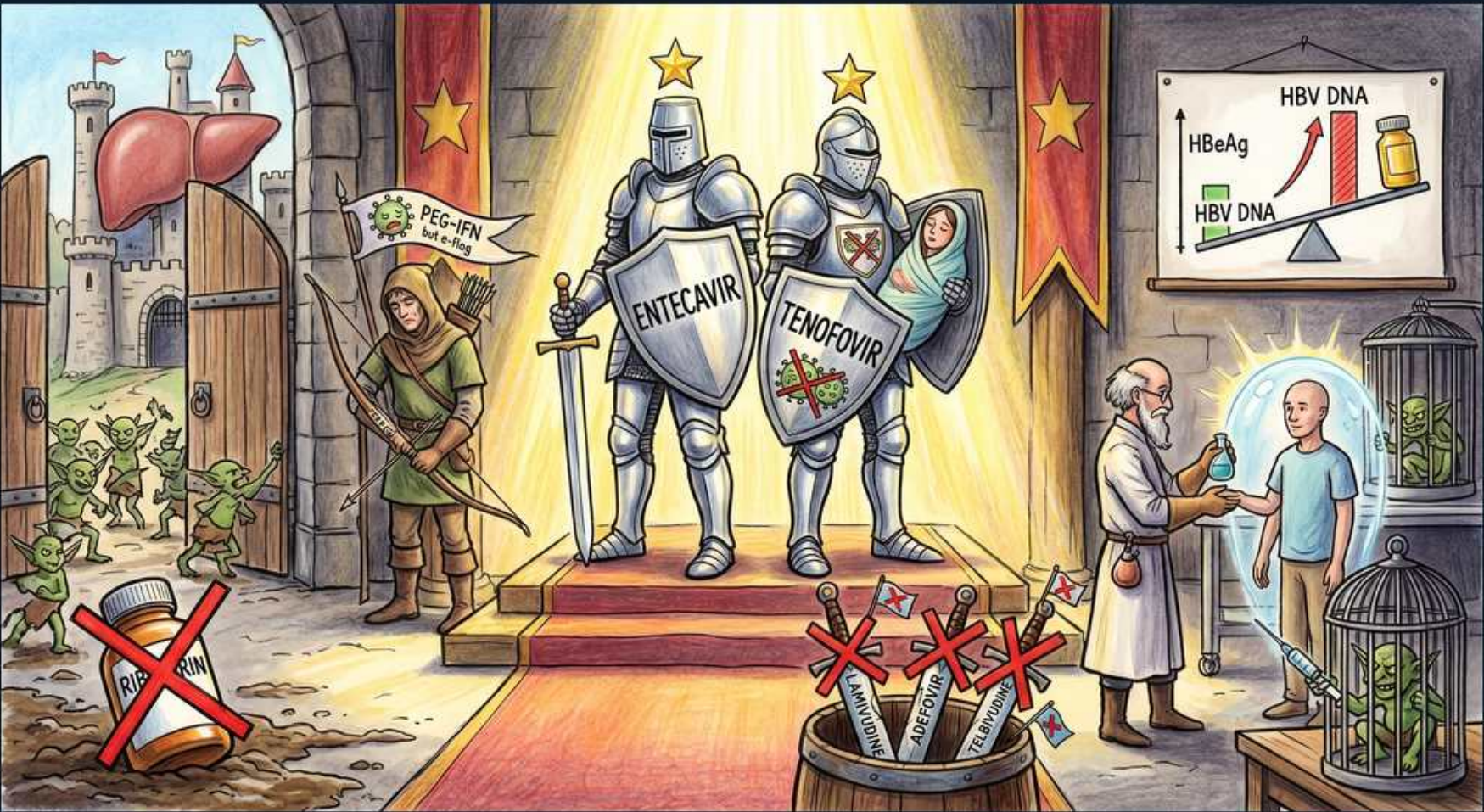


Chronic Hepatitis — HBV Treatment





1. Entecavir knight (gold star)

WHAT YOU SEE

Armored knight with gold-star shield reading 'ENTECAVIR', center-left.

WHAT IT ENCODES

GOLD STAR first-line: entecavir, a nucleoside analog with a HIGH barrier to resistance. Potent HBV DNA suppression, well tolerated. Dose-adjust in renal impairment; avoid as monotherapy if prior lamivudine resistance (cross-resistance).

2. Tenofovir knight (gold star)

WHAT YOU SEE

Knight with gold-star shield 'TENOFVIR', cradling a pregnant/baby figure, center.

WHAT IT ENCODES

GOLD STAR first-line: tenofovir (TDF/TAF), high barrier to resistance. PREFERRED in pregnancy (lowers vertical transmission) and in HIV co-infection (active against both, part of ART). TDF: monitor renal function and bone density; TAF is gentler on kidney/bone.

3. PEG-interferon archer

WHAT YOU SEE

Robin-Hood archer with 'PEG-IFN' banner, left.

WHAT IT ENCODES

PEG-interferon = a FINITE course (~48 weeks) aiming at HBeAg seroconversion and HBsAg loss; immunomodulatory, no resistance. CONTRAINDICATED in decompensated cirrhosis, autoimmune disease, pregnancy, severe psychiatric disease. Flu-like effects, cytopenias, depression.

4. Goblins escaping castle (decompensated)

WHAT YOU SEE

Green goblins fleeing the liver-flag castle, far left.

WHAT IT ENCODES

The breached castle = decompensated cirrhosis, where interferon is contraindicated (can precipitate liver failure/sepsis). Use a nucleos(t)ide analog (entecavir/tenofovir) instead, which is safe and indicated in decompensated disease.

5. Reactivation prophylaxis scientist

WHAT YOU SEE

Scientist giving a potion to a bald (chemo) patient who glows/strengthens, right.

WHAT IT ENCODES

Before chemotherapy, immunosuppression, rituximab, or high-dose steroids, screen HBsAg AND anti-HBc; if positive, give prophylactic entecavir/tenofovir to prevent reactivation. Start before and continue 6–12 months after immunosuppression. Rituximab/anthracyclines are the highest-risk.

6. Caged virus suppressed

WHAT YOU SEE

Virus/plant locked in a cage beside the patient, far right.

WHAT IT ENCODES

Prophylaxis keeps the virus 'caged' — suppressed HBV DNA so immunosuppression doesn't unleash a flare/fulminant reactivation. Skipping prophylaxis in an anti-HBc-positive patient on rituximab can cause fatal hepatitis.

BOARD TRAP

'Start chemo/rituximab without HBV prophylaxis in an anti-HBc-positive patient' → wrong; omitting entecavir/tenofovir prophylaxis risks fatal HBV reactivation.

7. HBeAg / HBV DNA balance chart

WHAT YOU SEE

Scale chart 'HBeAg' and rising-arrow 'HBV DNA', top right.

WHAT IT ENCODES

Treatment goals: suppress HBV DNA, achieve HBeAg→anti-HBe seroconversion, normalize ALT, and ideally HBsAg loss (functional cure, uncommon). Falling DNA on the scale = successful nucleos(t)ide suppression.

8. Ribavirin barrel (red X)

WHAT YOU SEE

Brown 'RIBAVIRIN' barrel with a big red X, bottom left.

WHAT IT ENCODES

Ribavirin is an HCV/RSV antiviral with NO role in HBV. Selecting it for HBV is a wrong-disease error. HBV = entecavir/tenofovir (± PEG-IFN); HCV = direct-acting antivirals (ribavirin only as a legacy add-on there).

BOARD TRAP

'Treat chronic HBV with ribavirin' → wrong; ribavirin is for HCV — HBV first-line is entecavir or tenofovir.

9. Old low-barrier swords (red X)

WHAT YOU SEE

Barrel of broken swords with red Xs labeled lamivudine / adefovir / telbivudine, bottom center.

WHAT IT ENCODES

Lamivudine, adefovir, and telbivudine are OLD low-barrier agents → high rates of resistance with prolonged use; no longer first-line. Entecavir/tenofovir replaced them. Lamivudine resistance also compromises later entecavir.

BOARD TRAP

'Use lamivudine (or adefovir/telbivudine) first-line for chronic HBV' → wrong; low genetic barrier breeds resistance — use entecavir or tenofovir.

10. HCC surveillance (US ± AFP)

WHAT YOU SEE

Implied monitoring of the at-risk patient on the right (surveillance context).

WHAT IT ENCODES

Chronic HBV needs HCC surveillance — abdominal ultrasound ± AFP every 6 months in cirrhotics and high-risk non-cirrhotics (Asian men >40, Asian women >50, Africans, family history of HCC). HBV causes HCC even without cirrhosis via direct integration.

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